



Where R we, and where R we going?

Results of the R-HTA Expert Elicitation Survey

R-HTA | 23 June 2026

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Was very sorry to miss the event yesterday



Rory



2 Chips-N-Bins Question

What percentage of all early health economic models built by global pharmaceutical companies and their supporting consultancies were built in the R programming language in the past 12 months?

Where early health economic models are defined as models used to guide internal decision-making but **not** used for submission to HTA bodies. Count any model where the primary computational engine or core simulation logic is executed in R or via R (e.g. Rcpp).

3 Chips-N-Bins Question

What percentage of health economic models for HTA/reimbursement submissions were built using R in the past 12 months?

4 Low-High-Best Question

In 2026, what percentage of practicing health economists in the industry possess the technical capability to independently code and validate a submission-ready model in R?

5 Low-High-Best Question

Looking ahead to the year 2030, what percentage of practicing health economists will possess the technical capability to independently code and validate a submission-ready model in R?

6 Competing Probability Question

What do you consider to be the primary barriers preventing the widespread adoption of R within health economics?

Allocate 100 points across the following barriers to reflect their relative impact in preventing the adoption of R



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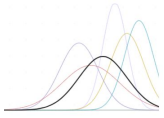
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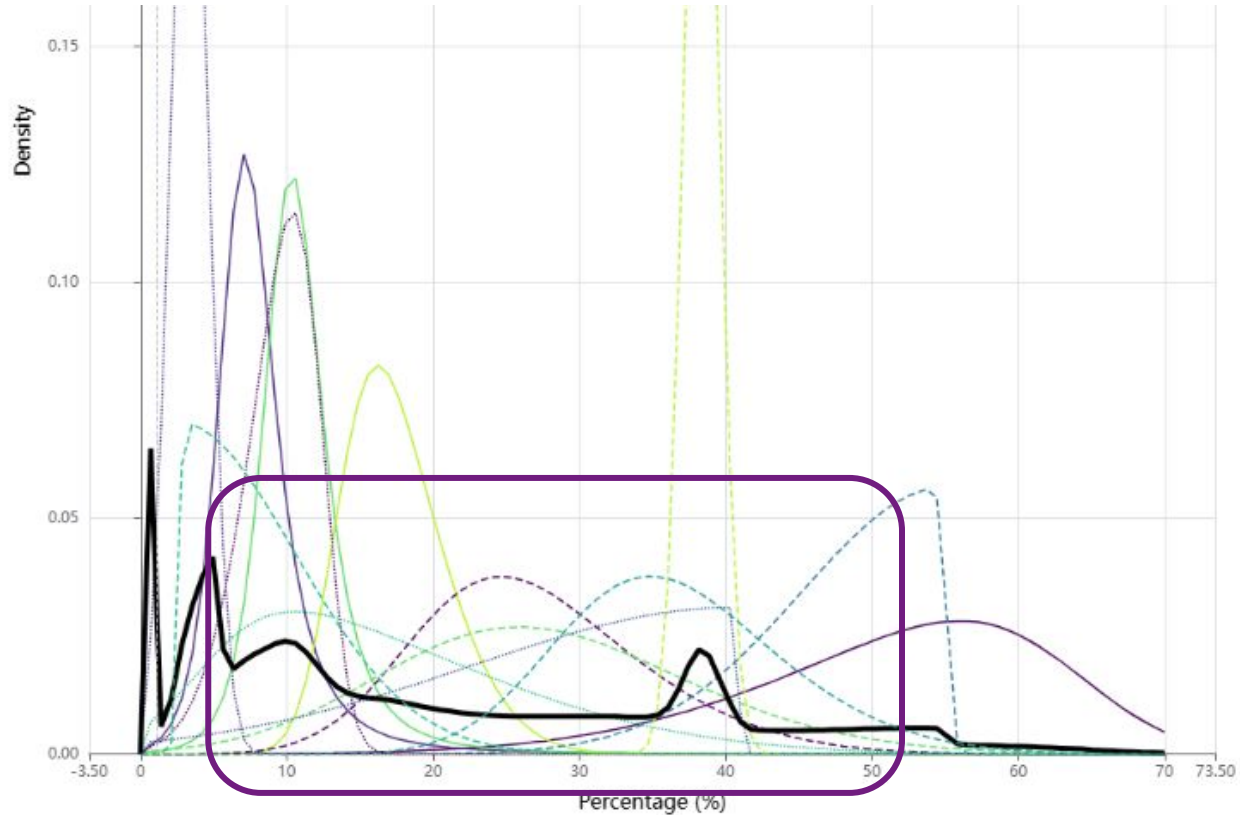
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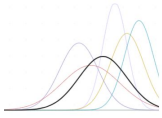
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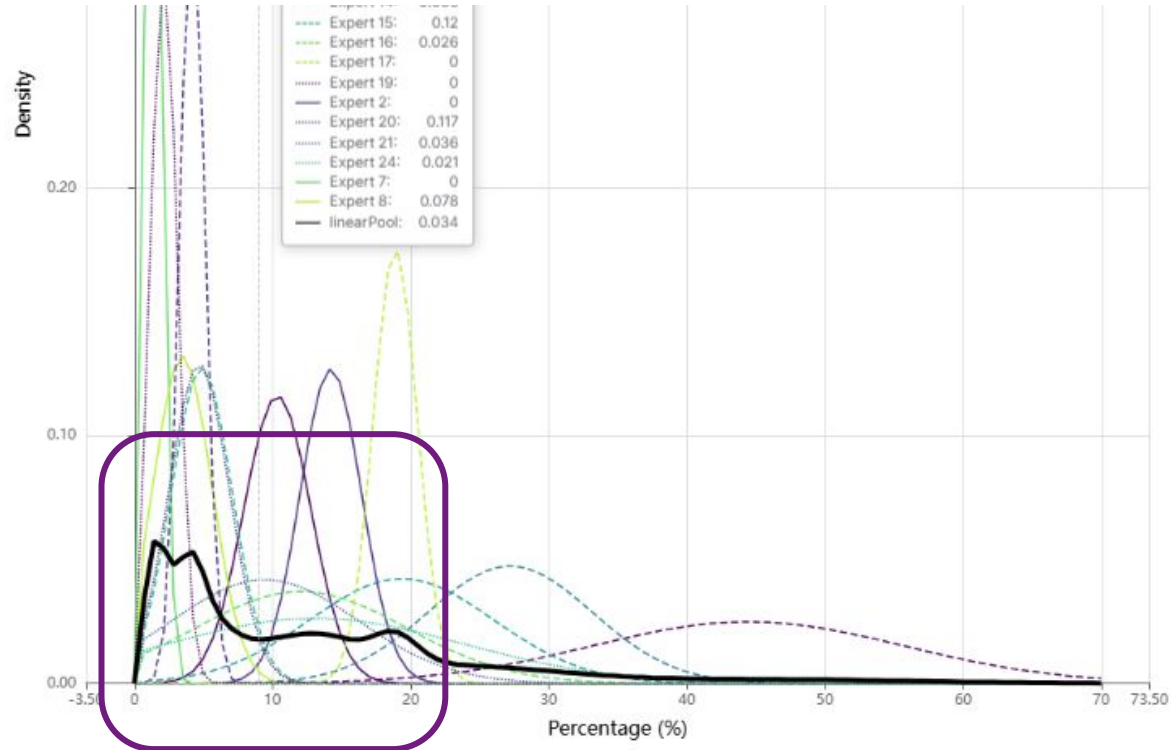


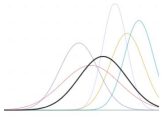
Early Economic Models (5-50%)





HTA models (0-20%)

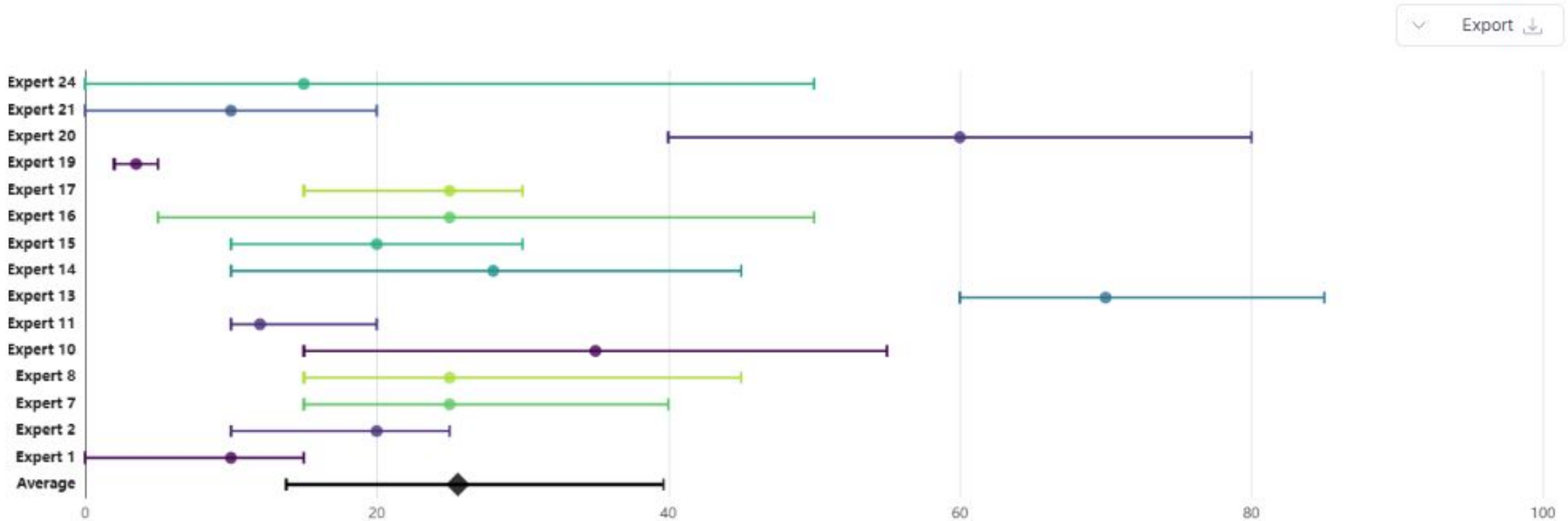


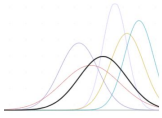


Question 3 of 5



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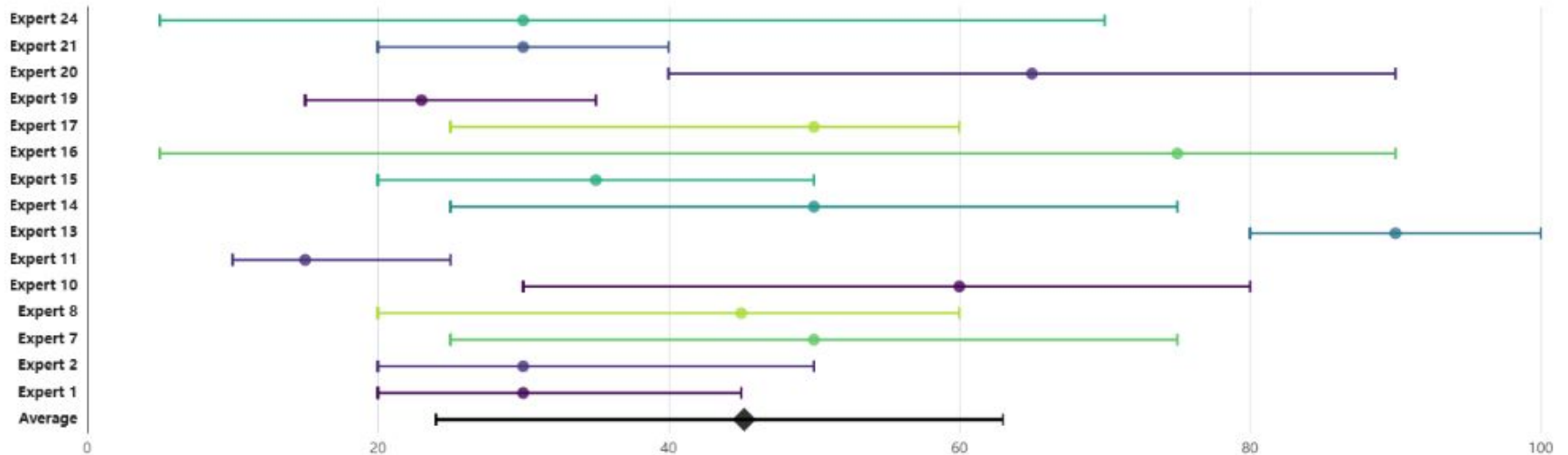


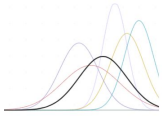
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Export





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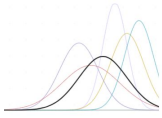
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The main drivers identified were:

- **AI and LLMs**, which are expected to make coding, learning and model development more accessible and efficient.
- **Growing use of R and scripted workflows** in HEOR, including reproducibility requirements, automated pipelines and reference models developed in R.
- **Improved training and generational change**, with newer entrants to the profession more likely to have coding skills.
- The potential for **R-based models to become the core model**, with Excel outputs or interfaces derived from them.

However, several experts cautioned that widespread coding adoption will not necessarily translate into universal ability to create and validate **submission-ready** models. Advanced technical judgement, validation skills and critical thinking will remain important, while some organisations and professionals may continue to use Excel, TreeAge or hybrid approaches. The broad and global definition of “practising health economists” was also seen as a reason not to expect complete uptake.



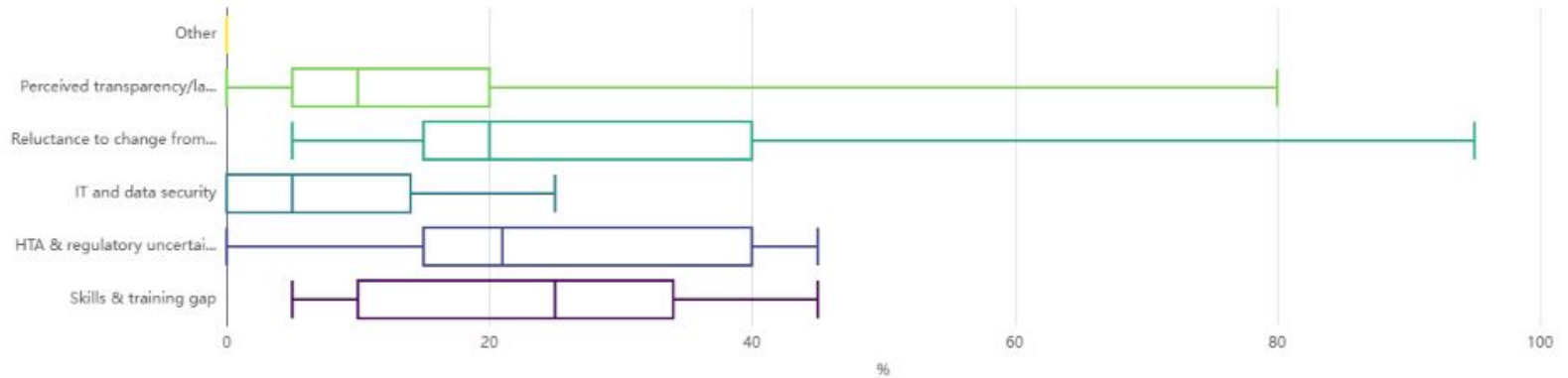
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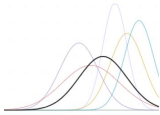
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Key barriers to wider R adoption

- **Skills gap:** Most health economists, reviewers and decision-makers are more familiar with Excel than R, reinforcing a perception that Excel is easier to interpret and more transparent.
- **HTA acceptability:** Inconsistent acceptance across agencies and markets discourages use of R for global models.
- **Organisational inertia:** Established Excel/TreeAge workflows, particularly among senior staff, are difficult to replace.
- **Need for clear practical benefits:** R uptake is more likely where it demonstrably improves reporting, testing, speed, reliability and model adaptation.
- **Secondary barriers:** Usability, interfaces, IT/data-security requirements and maintenance concerns.